

HOW TO OPEN A GROUP HOME BUSINESS in New Mexico

Family Living & Supported Living under the DD Waiver — serving adults with developmental disabilities.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New Mexico model — read this first

“Group home” in New Mexico means providing residential support to adults with intellectual and developmental disabilities, and the state funds it through the Developmental Disabilities (DD) Waiver, administered by the Developmental Disabilities Supports Division (DDSD) — now part of the New Mexico Health Care Authority (HCA). The residential piece is called LIVING SUPPORTS, and it comes in a few forms. Family Living is the host-home model: an adult lives with a paid caregiver — in your home or theirs — with no more than two people served at a time. Supported Living is the group-home model: a home or apartment of three or fewer adults who receive 24-hour support, priced in Categories 1 through 4 by level of need. (A third form, Intensive Medical Living, is for people who need daily skilled nursing.) Both Family Living and Supported Living are delivered by agencies contracted with the state.

***The two ways in.** Path A — become a Family Living provider under an existing DD Waiver agency: one or two adults live with you, the agency approves your home with a home study, supports you, and handles the billing. Fastest, simplest — the closest thing to a host home. Path B — become your OWN DD Waiver provider agency: you contract with DDSD, meet the agency requirements (insurance, bond, accreditation), and run Supported Living homes and/or recruit Family Living homes. A much bigger lift — but it's the real business. This guide walks both, and is honest about which is realistic when.*

***The names you'll deal with.** DDSD = Developmental Disabilities Supports Division (runs the DD Waiver, enrolls providers, sets the standards) — part of the Health Care Authority. HCA = the Health Care Authority, New Mexico's Medicaid agency (determines eligibility and pays claims). DDW = the DD Waiver, the Medicaid HCBS waiver that funds these services (Mi Via is New Mexico's self-directed alternative). Case Manager = develops the Individual Service Plan (ISP) and handles budget approvals — the person chooses their own. DDSD Regional Office = direct oversight and technical assistance in your area. DHI / QMB = the Division of Health Improvement and its Quality Management Bureau, which survey living-supports agencies (unannounced) for health, safety, and compliance.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change, so verify each step with DDSD, your DDSD Regional Office, the Division of Health Improvement, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start with a Family Living home or build your own agency.

1	Decide your path & set up the business	Path A (Family Living) or Path B (your own agency); entity, EIN, NPI, insurance (Parts 3–4)
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2	Learn the DD Waiver standards	Read the DD Waiver Service Standards and the Living Supports definitions (Part 5)
3	Become an approved provider	Family Living home study, OR the DDW Qualified Provider Application to DDS (Part 5)
4	Sign your DDS contract & enroll	Provider Agreement with DDS and New Mexico Medicaid enrollment (Part 5)
5	Ready the home	Living-supports settings, HCBS rights, fire safety, and DHI surveys (Part 6)
6	Staff & write your policies	Direct support personnel, RN oversight, documentation, Therap (Parts 7–8)
7	Get a person placed	Central Registry, allocation, SIS, ISP, the match (Part 9)
8	Deliver services & get paid	Bill NM Medicaid against the approved ISP budget (Part 10)

Rule of thumb. Start smaller than you think. Family Living — one or two adults living with you under an established agency — is the fastest, lowest-cost way in, and it teaches you the whole system on someone else's contract. Many providers begin as a Family Living home, learn how the DD Waiver actually works, and grow into their own DDS-contracted agency running Supported Living homes.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid, not a real-estate play or a normal small business: your revenue depends on being approved or contracted, then on a person choosing you — so your reputation with DDS, your Regional Office, established providers, and Case Managers is your most valuable asset. Second — the heads-up unique to New Mexico — the DD Waiver has a very long wait: a person can spend many years on the Central Registry before an allocation. But people are served every day, providers are always needed, and the money still arrives on a chain and on a delay — the home study or the Qualified Provider Application, the DDS contract, and Medicaid enrollment all take time, and you don't bill until a person is placed and their ISP budget is approved. You need patience and a working-capital cushion. Start on Path A as a Family Living home under an established agency if you can, learn the system on someone else's contract, and grow into your own agency once you know the terrain.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in; Part 5 is the approval-contract-enrollment chain; Part 6 covers the home, DHI surveys, and fire safety; Parts 7–8 are staffing and the policies and records the state expects; Part 9 is how a person actually gets placed with you; Part 10 is how you get paid; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential support (Living Supports) to adults with developmental disabilities under the DD Waiver — as a Family Living home (1–2 people, live-with) or a Supported Living home (3 or fewer, 24-hour). Paid by New Mexico Medicaid, after DDS approves or contracts with you.*

A “group home” business in New Mexico is really a developmental-disabilities support business. You help an adult with an intellectual or developmental disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. New Mexico’s residential waiver service for this is Living Supports, and it spans a spectrum: from supporting one or two people in a Family Living home (yours or theirs), to a Supported Living home or apartment of three or fewer adults with 24-hour support, to Intensive Medical Living for people who need daily skilled nursing.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid — not a real-estate play. Your value is the quality and reliability of the support you provide and your ability to keep it compliant and well-documented, because that’s what the home study or the provider standards require, what the ISP budget authorizes and pays for, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based services for people with developmental disabilities is high and durable: New Mexico, like every state, is moving people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and the DD Waiver funds exactly this. Because the person has freedom of choice and chooses their own provider, a reliable, well-run home that builds relationships with Case Managers can keep its settings full. And you can start lean — supporting one person in a Family Living home under an established agency — and grow into your own DDS-contracted agency as you learn the system.

Be honest about the two realities that trip up new providers. (1) *The wait is long: the Central Registry can take many years, so placements follow the state’s allocation schedule, not your timeline — provider demand is steady, but you serve one allocated person at a time.* (2) *Room and board is separate from your pay: the waiver pays for the SERVICE, not the person’s rent and food, which come from their own income (usually SSI).* Plan your business — and explain it to families — around both.

PART 2

Who Regulates You & Who Pays You

In short: DDSD administers the DD Waiver and approves or contracts with providers; HCA (Medicaid) determines eligibility and pays claims; and DHI/QMB surveys your home for health and safety. The person's Case Manager builds the ISP that authorizes what you deliver.

DDSD — Developmental Disabilities Supports Division (your administrator)

DDSD administers the DD Waiver, sets the DD Waiver Service Standards, enrolls provider agencies, and — through its Regional Office Bureau — provides oversight and technical assistance in your area. It approves the geographic area(s) and specific service(s) you're authorized to provide. Now part of the New Mexico Health Care Authority, DDSD is the center of gravity for everything below; you'll work with its Provider Enrollment & Relations Unit and your DDSD Regional Office throughout.

HCA — Health Care Authority / New Mexico Medicaid (your payer)

The Health Care Authority is New Mexico's Medicaid agency. It determines a person's Medicaid eligibility and pays claims for waiver services. As a provider you complete New Mexico Medicaid enrollment; a Family Living home's claims are filed by its provider agency, and a contracted agency bills Medicaid directly against each person's approved ISP budget.

DHI / QMB — Division of Health Improvement (your surveyor)

New Mexico doesn't license these homes as facilities. Instead, the Division of Health Improvement and its Quality Management Bureau conduct unannounced surveys of living-supports agencies for health, safety, and compliance with the DD Waiver Service Standards. So you're meeting standards and passing surveys rather than getting a facility license. Get to know the survey field tools early and self-assess before you open.

What this means for you: the path to payment runs through more than one door — DDSD (approve/contract), HCA/Medicaid (enroll and pay), and DHI/QMB (survey) — and each takes time. Start them in the right order, and treat your DDSD Regional Office and the Case Managers in your area as key relationships, because the person chooses their own provider. A home that's approved or contracted, enrolled, survey-ready, and known to Case Managers is a home that fills.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New Mexico you form the LLC through the Secretary of State. Your entity, EIN, and NPI all have to be in place and consistent before your DDS provider application and Medicaid enrollment, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the New Mexico Secretary of State.** Online at the SOS business portal — the domestic LLC formation fee is about \$50, filed online. Name the LLC and list a registered agent with a physical New Mexico address (this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your DDW Qualified Provider Application, and Medicaid enrollment will reference your structure and board/ownership list.
- 3. Know New Mexico's light ongoing burden.** New Mexico LLCs file no annual report and pay no annual LLC report fee to the Secretary of State — a genuine simplification compared with many states. Keep your registered agent and address current.
- 4. Register with Taxation & Revenue and confirm local licenses.** Register with the NM Taxation & Revenue Department for your Business Tax Identification / gross-receipts account, and check what city or county business licenses apply where you'll operate.

No newspaper publication in New Mexico. New Mexico has NO LLC publication requirement and no annual report — forming the entity is a straightforward Secretary of State filing (about \$50) with very little to maintain year to year. That's one of the easier parts of this whole process.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to complete New Mexico Medicaid enrollment tied to DDS and to bill the DD Waiver for Living Supports. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a developmental-disabilities / community-based residential provider — confirm the exact one DDSD and New Mexico Medicaid want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of New Mexico branches — Nusenda Credit Union, Bank of Albuquerque (BOK Financial), Wells Fargo, U.S. Bank, Washington Federal (WaFd), or a local community bank or credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance & bonding — what you need, who sells it, and what to ask

A living-supports provider brings staff into people's homes and supports vulnerable adults, so you need coverage built for a human-services provider — and DDSD (which names the Health Care Authority as an additional insured) will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the SUPPORT and supervision you provide	Yes — DDSD requires \$1M min.
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Surety / Fidelity Bond	Protects against theft/dishonesty — DDSD requires a bond (see 3.6)	Yes — \$10,000 min. (agency)
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	DSPs driving their own cars to support or transport the people you serve	Yes — critical
Workers' Compensation	DSP injuries	Required in NM (3+ employees)

Abuse & molestation is your highest exposure. Supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. And name the Health Care Authority as an additional insured on your general/professional liability, as DDSD requires. Confirm both explicitly.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local New

Mexico agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a developmental-disabilities / DD-Waiver provider program. Confirm the carrier writes this in New Mexico before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities / living-supports providers in New Mexico — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Can you provide the \$1,000,000 general/professional liability with the Health Care Authority named as additional insured, and the required surety/fidelity bond?”

“Do you include non-owned & hired auto for my DSPs driving the people we support?”

“Can you set up New Mexico workers' comp for my direct support personnel?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves New Mexico providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your DDW Qualified Provider Application, your DDSD Provider Agreement, and your New Mexico Medicaid enrollment. Your information passes through more than one system before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled enrollment or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your contract, enrollment, insurance, bond, and any accreditation calendared for renewal — letting any one of them lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply to become an agency:

- Your formed New Mexico LLC (Articles of Organization filed) and Operating Agreement, plus Articles of Incorporation and board list if you're a corporation
- Your EIN confirmation (CP-575) from the IRS
- Your NM Taxation & Revenue registration (CRS / Business Tax Identification)
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance (\$1M liability, HCA as additional insured) and the required surety/fidelity bond
- A single, consistent legal name, ownership list, and address used identically on every document

With those things in place and consistent, you're ready to build the provider application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more New Mexico applications than any substantive problem does.

PART 4

The Two Paths — Family Living (A) or Your Own Agency (B)

In short: Path A is the fast, small way in — a Family Living home under an established DD Waiver agency. Path B is the real business — become your own DDS-contracted provider agency running Supported Living homes. Pick the one that fits where you are today; many start with A and grow into B.

4.1 Path A — a Family Living home under a DDW agency

Family Living means one or two adults with developmental disabilities live with you and share your everyday life — in your home or theirs — with no more than two people served at a time. On this path you don't contract with the state yourself; you partner with a DDS-contracted Family Living provider agency. It's the fastest way in and the closest thing to a host home. New Mexico has roughly 70 waiver provider agencies — ask around your DDS Regional Office and interview more than one.

- 1. Make sure it fits your life.** Everyone in the household is part of the approval process, so it's a family decision — go in with eyes open about the good days and the hard ones.
- 2. Choose a DD Waiver provider agency.** Partner with a DDS-contracted Family Living agency: they complete your approval, support and monitor you, arrange substitute coverage so you can take a break, and handle the billing. Interview more than one.
- 3. Complete your home study & training.** The agency completes a DDS-approved home study of your home and family before anyone is placed, plus required training. The home study is updated every year and any time your family composition changes or you move — this is the heart of Family Living approval.
- 4. Pass background checks, then get matched.** You and the adults in your home complete the required criminal-history and abuse-registry checks (New Mexico screens against its caregiver registry). Then the agency, the person's Case Manager, and the team match you with someone whose needs and personality fit your home.

The difficulty-of-care tax note. Because the person lives in your OWN home, your Family Living payments may qualify for the IRS “difficulty-of-care” exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of a Family Living arrangement.

4.2 Path B — become your own DD Waiver provider agency

Path B is more work, more money, and more control: you contract with DDS, meet the agency requirements (insurance, bond, accreditation), enroll with New Mexico Medicaid, and run Supported Living homes and/or recruit Family Living homes. It's the real, scalable business — and Part 5 walks the whole path step by step. Most operators who go this route either have direct experience in DD services or hire someone who does, because your DDW Qualified Provider Application has to show DDS you understand the standards and can deliver.

PART 5

Become an Approved Provider — Home Study (A) or Qualified Provider Agency (B)

In short: For Path A, approval is the agency's home study and training. For Path B, it's the chain that unlocks payment: learn the standards, line up insurance/bond/accreditation, submit the DDW Qualified Provider Application to DDSD, sign your Provider Agreement, and enroll with New Mexico Medicaid.

5.1 Path A — the Family Living home study

On Path A the approval is simpler and the agency carries most of it: a DDSD-approved home study of your home and family, the required training, and background checks (covered in Part 4). The home study is updated annually and whenever your family or home changes. You don't contract with the state or enroll with Medicaid yourself — the agency does, and bills on your behalf.

5.2 Path B — the Qualified Provider Agency chain

- 1. Learn the standards cold.** Read the DD Waiver Service Standards (Family Living, Supported Living, and Intensive Medical Living definitions and standards) and the Living Supports rules closely — your application and policies must reflect them and describe HOW you'll deliver services.
- 2. Line up insurance, a bond, and accreditation.** General or professional liability at a \$1,000,000 minimum, naming the Health Care Authority as an additional insured; a surety bond (individual) or fidelity bond (group) at a \$10,000 minimum per DDSD requirements; and national accreditation (for example CARF or CQL) — or a DDSD accreditation waiver for your initial agreement term.
- 3. Apply to become a DDW Qualified Provider Agency.** Submit the DDW Qualified Provider Application to DDSD's Provider Enrollment & Relations Unit — including your Articles of Incorporation and board list, CRS certificate, insurance and bond, an organizational chart with position descriptions, and answers to any service-specific questions (for example, your nursing plan). DDSD approves the geographic area(s) and specific service(s) you're authorized to provide.
- 4. Sign your DDSD contract & enroll with Medicaid.** All provider agencies must have a Provider Agreement with DDSD and complete New Mexico Medicaid enrollment. Remember: waiver services aren't an entitlement and funding isn't guaranteed — you're authorized to serve within your approved areas and services.
- 5. Stand up your homes, policies & staff, then serve.** Build your Supported Living home(s) to the DD Waiver Service Standards, put your policies and procedures in place, hire and train your staff, and prepare for DHI/QMB surveys. Then you can accept referrals, deliver services, and bill (Parts 9 and 10).

What this means for you: becoming a Qualified Provider Agency is a quality gate, not a form. DDSD wants evidence — real policies, real service descriptions, real capacity, real insurance and a bond — that you can safely support people. The providers who get through cleanly treat the DD Waiver Service Standards as the blueprint for the whole operation, so building your policies and staffing to the standard does double duty: it earns your approval AND it's how you'll actually run.

PART 6

The Home — Settings, HCBS Rights, DHI Surveys & Fire Safety

In short: *New Mexico doesn't license these homes as facilities. A Family Living home is approved by a home study; a Supported Living home is a provider-operated residence held to the DD Waiver Service Standards. Both are surveyed (unannounced) by DHI/QMB. In every case, it must genuinely be the person's home.*

6.1 How New Mexico handles the home

A Family Living home is approved through a home study by your provider agency, and a Supported Living home is a provider-operated residence held to the DD Waiver Service Standards. Both are overseen by the Division of Health Improvement (DHI) and its Quality Management Bureau (QMB), which conduct unannounced surveys for health, safety, and compliance. So you're meeting standards and passing surveys, rather than getting a facility license. In every case, the home must genuinely be the person's home — with privacy, choice, and full access to the community under the federal HCBS Settings rule.

6.2 “Do I need fire sprinklers?” — the honest answer

Because Family Living (one or two people) and Supported Living (three or fewer) happen in small private or provider-operated residences — not large licensed facilities — commercial fire-sprinkler systems generally are NOT required. What you must have is a genuinely safe home that meets the DD Waiver health-and-safety standards and passes DHI surveys. Fire requirements come from your local fire authority and building code, so confirm anything unusual — especially for a larger or modified home — with your local fire authority and DDSD BEFORE you commit to a property.

6.3 The home checklist (DD Waiver Service Standards)

- ☐ Working smoke and carbon-monoxide detectors, a fire extinguisher, and clear, usable exits with a practiced evacuation plan.
- ☐ Safe hot-water temperature — DDSD sets water-temperature requirements for Supported Living, Family Living, and Intensive Medical Living to prevent scalding.
- ☐ Safe, clean, sanitary conditions — sound utilities and electrical, working plumbing, and safe food storage and preparation.
- ☐ Medications and any hazardous materials stored safely; individual rights and privacy respected — this is someone's home, under the HCBS Settings rule.
- ☐ For Family Living: an approved home study (updated annually and when your family or home changes). For Supported Living: compliance with the Service Standards and readiness for DHI/QMB surveys.

Two things people get wrong. (1) Zoning — a Family Living or small Supported Living home is a residence, and homes for people with disabilities are protected as residential use under the federal Fair Housing Act; still, confirm your address and any local rules with your city or county. (2) Room and board is separate from your pay — New Mexico does NOT pay room and board through the waiver; the person's room and board comes from their SSI or personal funds. Your Living Supports rate is for the service. Keep the two completely separate in your books and agreements.

PART 7

Who You Actually Need — Staffing

In short: *In Family Living (Path A), you and your household are the support, and the agency arranges substitute coverage so you can take time off — you don't build a staff. As a certified agency (Path B), plan for a director, direct support personnel, RN oversight, quality/compliance, and billing — one person can wear several hats at the start.*

7.1 The roles

Family Living (Path A): you and your household are the support. The agency provides the professional oversight, the home study, and the billing, and is responsible for arranging substitute coverage so you can take time off. You don't build a staff. For Supported Living / your own agency (Path B), plan for these roles:

- **Agency director / administrator:** runs the agency and owns compliance and the DDS contract.
- **Direct support personnel (DSP):** the caregivers who provide 24-hour support in your Supported Living homes — each screened and trained.
- **Registered Nurse:** for health oversight, medication, and (for Intensive Medical Living) direct skilled nursing; RNs/LPNs must meet the NM Nursing Practice Act.
- **Quality / compliance staff:** to keep up with the DD Waiver Service Standards and prepare for DHI/QMB surveys.
- **Billing / administrative support:** handles Medicaid billing, budgets, and documentation — New Mexico uses the Therap system for much documentation.

7.2 Recruiting and keeping DSPs — the real make-or-break

Direct support personnel are the heart of Supported Living and the hardest thing to keep. DSP turnover is high across the whole field, and a home that can't reliably staff its 24-hour shifts can't safely support people or pass its surveys — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required background and abuse-registry checks and training, and a competency check before a DSP supports someone alone — this protects the people you serve and satisfies DDS and DHI.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a DSP hits a hard day, and recognition. Every DSP who quits costs you re-hiring, re-training, and often continuity for the person they supported.

PART 8

Policies, Records & Staying Survey-Ready

In short: *Your policies and documentation are what the standards require and what the ISP budget pays against. Build them to the DD Waiver Service Standards from day one — medications, emergencies, incident and Abuse/Neglect/Exploitation reporting, and the rights of the people you support.*

You keep clear documentation covering medications, emergencies, incident reporting (including New Mexico's Abuse/Neglect/Exploitation reporting), and the rights of the person you support — and the Individual Service Plan drives what you deliver and record. Much of this lives in the Therap system New Mexico uses. This documentation is both the standard's requirement and the backbone of a safe, well-run operation. Build a simple system that keeps each person's ISP, service records, and incident documentation current, because DHI surveys it and audits check it.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Central Registry to Move-In

In short: *You can't get paid until someone is placed with you and their ISP budget is approved. The path runs from eligibility and the Central Registry, through an allocation and a SIS assessment, to the ISP, the budget, and the match — with the person freely choosing their provider.*

- 1. The person qualifies — and waits.** To receive the DD Waiver, a person must meet New Mexico's developmental-disability definition and an ICF/IID level of care, plus financial eligibility. Because slots are limited, most people spend years on the Central Registry before an allocation — a real feature of the New Mexico system.
- 2. They get allocated.** When funding is available, the person is allocated off the registry (crisis situations can be expedited by the DDSD Expedited Referral Review Team).
- 3. The plan and budget are built.** A Case Manager — the person's choice — develops the Individual Service Plan; a Supports Intensity Scale (SIS) assessment helps set support needs, and the budget, including your Supported Living category, is reviewed by the Medicaid third-party assessor.
- 4. The match.** The person chooses providers (freedom of choice); you or your agency are matched based on fit and your approved services and area.
- 5. Move-in.** With the ISP and budget approved, the person moves in and you're delivering a billable service.

What this means for you: because people choose their own provider and slots are limited, your census depends on relationships and reputation, not marketing. Get known to your DDSD Regional Office, to established providers, and above all to the Case Managers in your area as the provider who supports people well, communicates, and handles a placement smoothly — and the matches follow.

Plan your business around the Central Registry reality. The wait can take many years, and allocations come on the state's schedule — which shapes how you should plan. Don't build a business model that assumes you'll fill three beds the month you're contracted; assume a gradual build as Case Managers learn to trust you and matches come through. Two practical moves help: start on Path A as a Family Living home under an established agency so you have revenue while you build, and stay in regular, helpful contact with the Case Managers in your area so that when one of their people is choosing a provider, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one allocated person at a time.

PART 10

How You Get Paid: The Daily Rate & Your First Payment

In short: *Living Supports are paid as a daily rate through New Mexico Medicaid. Family Living has its own rate; Supported Living is priced in Categories 1–4 by level of need. What you can bill is set by the person's approved DD Waiver budget in their ISP. Room and board is separate.*

10.1 How the rate works

Living Supports are paid as a daily (per-diem) rate through New Mexico Medicaid. Family Living has its own rate; Supported Living is priced in four categories (Category 1 through Category 4) that reflect the person's assessed level of need and the staffing the home requires — a higher category means more support and a higher rate. The amount you can bill is set by the person's approved DD Waiver budget in their ISP.

- **Family Living vs. Supported Living:** separate rate models — Family Living (1–2 people, live-with) is priced differently from Supported Living (3 or fewer, 24-hour).
- **Supported Living Category (1–4):** the category, set from the person's assessed needs, drives the daily rate and the staffing the budget assumes.
- **Budget-approved:** your services are authorized in the person's ISP budget, reviewed by the Medicaid third-party assessor — you bill according to that approval.
- **Where to look:** the exact procedure codes and current per-diem rates are in the DD Waiver fee schedule / rate table (HCA). Confirm the current figure before you budget.

10.2 The money math — how Living Supports pencils out

Living Supports economics are different from an hourly private-pay business — your revenue is the daily rate for the authorized service, and your biggest cost is DSP labor. Lay it out with your own numbers:

- **Revenue:** the authorized Living Supports service in each person's ISP, billed to New Mexico Medicaid at the daily rate — Family Living's own rate, or the assigned Supported Living Category 1–4 rate.
- **Cost:** DSP wages and payroll taxes are the bulk in a Supported Living home (24-hour staffing); then RN oversight, administration, insurance and the bond, and the building. Most are fixed against your authorized census.
- **The lever:** because the daily rate is set by the waiver and driven by category, your margin comes from staffing efficiently to each person's assessed category and keeping your homes filled with allocated people — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the person's rent and food come from their own income (usually SSI), NOT the waiver — keep that money and those agreements entirely separate from your service billing.

A few New Mexico specifics shape the math. Because Supported Living is capped at three or fewer adults and Family Living at no more than two, you can't spread your fixed costs across a large census the way a big facility would — your model has to work at small numbers, which is exactly why staffing efficiency and keeping every authorized bed filled matter so much. The person's category isn't something you set; it flows from the SIS assessment and the Medicaid third-party assessor's budget review, so if you build a Supported Living home you should staff to the categories your ISPs actually authorize, not to what you hoped for. And because the daily rate is per-diem, a bed that sits empty between allocations is pure lost revenue against staffing you may still be carrying — one more reason the Central Registry and your Case Manager relationships (Part 9) drive the economics as much as the rate table does.

10.3 How you bill — and one rule to remember

- **Billed to New Mexico Medicaid:** claims go to NM Medicaid — a Family Living home's claims are filed by its provider agency; a contracted agency bills directly.

- **Room and board is separate:** never billed to the waiver, and never billed to the person as part of your service.
- **Documentation matters:** New Mexico requires specific billing and service documentation (much of it in the Therap system) — keep it current, because surveys and audits check it.

10.4 Your first payment

That first Medicaid payment is the moment everything you built clicks into place — the entity, the approval or contract, the enrollment, the home, and the person's approved ISP finally turn into revenue. Here's the sequence, start to finish:

1. **Confirm the ISP budget authorizes your service.** Check that the person's Individual Service Plan budget authorizes exactly the service you're providing — Family Living, or the right Supported Living category — before you deliver, because claims only pay against what the ISP approves.
2. **Deliver the service and keep your daily documentation.** Provide the support and record it as you go, much of it in the Therap system New Mexico uses — the daily documentation is what backs your claim and what surveys and audits check.
3. **Submit the claim to New Mexico Medicaid.** Your agency files the claim for a Family Living home; a contracted agency bills New Mexico Medicaid directly against the approved ISP budget.
4. **Get paid — and keep everything current.** Keep the budget, authorizations, insurance, bond, contract, and enrollment current, since claims pay against the approved ISP and any lapse stops payment even when the care is going perfectly.

Keep the two clocks in view. Two timelines run at once in New Mexico. The provider clock — approval or contract, enrollment, and readying the home — is under your control and takes weeks to months. The person clock — the Central Registry, allocation, the SIS, and the ISP — runs on the state's schedule and can take years. You get paid only when both line up, so build the provider side cleanly and completely, then let the placement come. That's the whole rhythm of getting paid on the DD Waiver.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: Path A (Family Living) can move relatively quickly; Path B (your own agency) takes longer through the application, contract, and enrollment. And the bigger clock is the Central Registry — placements follow the state's allocation schedule, so relationships with Case Managers are your census.

11.1 Timeline & startup costs

A Family Living home (Path A) can move relatively quickly once you've chosen an agency — the home study, training, and background checks. Becoming your own DD Waiver agency (Path B) takes longer: the business setup, insurance and bond, accreditation, the DDW Qualified Provider Application, your DDSD contract, and Medicaid enrollment — plan for several months end to end. And remember the bigger clock: because the Central Registry wait is long, provider demand is steady but placements follow the state's allocation schedule. Family Living costs are modest — mainly your time, background checks, training, and getting your home ready. A DD Waiver agency adds business formation (about \$50 with the Secretary of State and no annual report), the \$1M liability insurance and the surety/fidelity bond, accreditation, staff payroll, and your policies and procedures. Confirm current figures and timelines with DDSD and your DDSD Regional Office.

11.2 Getting your first placements

Placements don't come from advertising — they come through the state's allocation process and your reputation. Here's a simple plan:

- 1. Get known to your DDSD Regional Office and established providers.** Ask where the need is and which agencies to talk to; if you're starting on Path A, connect with established DDW agencies that bring on Family Living homes.
- 2. Build relationships with Case Managers.** They coordinate the ISP and the match, and the person chooses. Be the provider they trust to handle a placement well, communicate, and never leave a shift unstaffed.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your DDSD Regional Office is the center of gravity, with DHI and your local fire authority close behind. Statewide contacts are the same for everyone; your fire/building offices are local. Here's the list and the scripts.

12.1 Statewide (same for everyone)

For...	Contact
Becoming a provider (enrollment)	DDSD Provider Enrollment & Relations — hca.nm.gov (Developmental Disabilities Supports Division)
Waiver eligibility, allocation & the registry	DDSD Pre-Service Intake Bureau / your DDSD Regional Office
Health & safety oversight (surveys)	Division of Health Improvement (DHI) / Quality Management Bureau (QMB)
Rates, fee schedule & billing	New Mexico Medicaid (HCA) — DD Waiver fee schedule / rate table
Report abuse, neglect or exploitation	ANE Hotline — 866-654-3219
Business, EIN & NPI	sos.nm.gov · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① Your DDSD Regional Office or a DDW agency — your most important calls

"I want to become a Family Living provider / start a Supported Living agency in this area. Where's the need, and which agencies should I talk to?"

"(To an agency) How do you approve and support Family Living homes, how do you match, and how and when do you pay?"

② Division of Health Improvement — the surveys

"What do your living-supports surveys check, and where can I get the survey field tools to self-assess my home before I open?"

③ Local fire authority / building — the home

"I'll operate a small residential home for 1–3 adults with developmental disabilities. What fire-safety requirements apply, and do I need anything beyond smoke/CO detectors and extinguishers?"

12.3 Worked examples — New Mexico's two biggest metros

Most people start in or near one of these two. DDSD runs seven regional offices statewide (Metro/Albuquerque, Santa Fe, Taos, Gallup, Farmington, Las Cruces, Roswell) — find yours by where you live.

- **Albuquerque (DDSD Metro Region):** DDSD Regional Office — Metro Office, 5300 Homestead Road NE, Albuquerque, NM 87110 (covers the Albuquerque metro area); DDSD Provider Enrollment & Relations (via hca.nm.gov) for enrollment; City of Albuquerque / Bernalillo County fire authority for fire safety; DHI/QMB for the living-supports survey. The quirk: the Metro region is the state's largest and busiest — build relationships with local Case Managers, since the person chooses their provider.
- **Las Cruces (DDSD Southwest Region):** DDSD Regional Office — Southwest Office, 1170 N. Solano Drive, Suite G, Las Cruces, NM 88001 (covers the southwest, including Doña Ana County); the same statewide DDSD process for enrollment; City of Las Cruces / Doña Ana County fire authority; DHI/QMB for the survey. The quirk: the southwest region is large and rural beyond Las Cruces — confirm which communities your approved service area covers.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to become my own agency, or can I start smaller?

You can start smaller — Path A lets you become a Family Living home under an established DD Waiver agency that handles your approval and billing. It's the fastest way in and the best way to learn the system before you take on your own DDSD contract (Path B). Many providers grow from A to B.

Why does the wait matter so much?

New Mexico's DD Waiver has a very long wait — the Central Registry can take many years before a person is allocated a slot. That doesn't mean people aren't served; they're served every day, and providers are always needed. But it means your census builds on the state's allocation schedule, so plan for a gradual ramp and build relationships with Case Managers, since the person chooses their provider.

Does the waiver pay for the person's rent and food?

No — New Mexico does NOT pay room and board through the waiver. The waiver pays for the SERVICE (your Living Supports), and the person's rent and food come from their own income, usually SSI. Keep the two completely separate in your agreements and your books; mixing them is a common and serious mistake.

What's the difference between Family Living, Supported Living, and Intensive Medical Living?

Family Living is the host-home model — one or two adults live with a paid caregiver. Supported Living is the group-home model — a home or apartment of three or fewer adults with 24-hour support, priced in Categories 1–4 by level of need. Intensive Medical Living is for people who need daily skilled nursing. All three are Living Supports under the DD Waiver, delivered by state-contracted agencies.

You can do this

Opening a group home business in New Mexico is real work — the waiver system has a lot of moving parts, and the wait for services is long — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start with a Family Living home or build your own agency, take it one step at a time: set up the business, learn the DD Waiver Service Standards, get approved or contracted and enrolled, ready the home, and build the relationships that bring you placements. Verify the current details with the State as you go, and lean on your DDSD Regional Office — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in New Mexico DDSD / Health Care Authority rules, the DD Waiver Service Standards, and the New Mexico Administrative Code (8.314.5 NMAC — Living Supports; 8.290.4 / 8.313.2 NMAC — eligibility and level of care) as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with DDSD, the Health Care Authority, DHI, and qualified counsel before you act.